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 / **Practice Exam - 1**

Correct Answer

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## Practice Exam - 1

### Question 45 of 117



A 64-year-old man with nonischemic cardiomyopathy (LVEF 20%) and chronic HFrEF is seen for routine follow-up. He reports progressive exertional dyspnea and fatigue over the past 4 months. He can walk one block slowly on level ground but becomes symptomatic at a normal pace or when climbing a single flight of stairs. He becomes dyspneic while showering and must stop to rest midway through dressing. He denies dyspnea at rest, orthopnea, or paroxysmal nocturnal dyspnea. He has had no hospitalizations or emergency department visits in the past 6 months. His medications include sacubitril-valsartan 24-26 mg bid, metoprolol succinate 25 mg a day, spironolactone 25 mg, and dapagliflozin 10 mg. On examination, blood pressure is 98/62 mm Hg, heart rate 82/min, and oxygen saturation 97% on room air. Jugular venous pressure is 8 cm H<sub>2</sub>O. There are no crackles. Cardiac exam reveals an S3 gallop. There is no ankle edema. Extremities are warm. Laboratory testing shows stable renal function (Cr 1.4) and an NT-proBNP similar to prior values.

Which INTERMACS profile best describes this patient's current clinical status?

A. Profile 1



B. Profile 2

C. Profile 3



D. Profile 4

E. Profile 5

#### Commentary

References

**Correct Answer:** D. Profile 4

#### **Teaching Point:**

The board-certified AHFTC specialist should understand the clinical application of NYHA, ACC/AHA staging, and INTERMACS profiles for assessment of advanced heart failure severity and referral timing for advanced therapies.

**Explanation:**

INTERMACS profile 4 (“Resting symptoms” / advanced NYHA III–IV but stable) describes patients with advanced heart failure who are at home on oral therapy but have symptoms with activities of daily living and evidence of progressive functional limitation.

**Incorrect Answers:**

Profile 1. “Critical cardiogenic shock”. No evidence of overt shock

Profile 2. Progressive decline on inotropes/Inotropes sliding. Patient is not on inotropes and deteriorating

Profile 3 Stable but inotrope dependent. Patient is not on inotropes

Profile 5. “Exertion intolerant”. Patient is comfortable at rest and with basic activities of daily living but cannot do meaningful exertion.

This patient has symptoms with ADL including showering.

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